This Patient Group Direction (PGD)is intended only for registered healthcare professionals who have been named and authorised by their organisation to practice under it.

Patient Group Direction
for the administration of Shingrix for the prevention of Shingles

Patient Group Direction, version 010, issued 30 September 2026. Supersedes Version 009, 14 September 2026.

Owner and authorising organisation: Get Real Health Limited, company number 12744898, registered office Zenvite Health Unit 23, St. Asaph Business Park, St. Asaph, Denbighshire, LL17 0LJ. This Patient Group Direction is authorised on behalf of Get Real Health Limited by its Medical Director and its Head Pharmacist, whose signatures for this version are on the authorisation page at the end of the document. It applies to the registered healthcare professionals named under it by a pharmacy or clinic that has adopted it through a Get Real Health service agreement, at the premises stated in that adoption, and to no other organisation.

Summary of the governing guidance for shingles vaccination: Green Book chapter 28a (Shingles), the JCVI advice of 2021 that moved the programme to Shingrix, and the Shingrix SmPC. NICE CKS 'Shingles' summarises the same programme.

Purpose of Vaccination

- To **prevent shingles (herpes zoster)** and its complications, particularly **postherpetic neuralgia (PHN)**.
- Risk of shingles increases with **age** and **weakened immunity**.

Vaccines Available

Shingrix is the only shingles vaccine in use in the UK programme. Zostavax (live attenuated) was withdrawn from the programme on 1 September 2023, is no longer supplied, and is not authorised under this PGD.

1. **Shingrix® (recombinant, non-live)**
    - **Preferred vaccine**, especially for immunocompromised individuals.
    - Licensed for adults aged 50 years and older, and for adults aged 18 years and older at increased risk of shingles. This PGD covers adults aged 50 and over (Arm 1) and adults aged 18 to 49 who are severely immunosuppressed as defined in Green Book chapter 28a, Box 1 (Arm 2).
    - Administered **intramuscularly**.

Eligibility (UK Immunisation Schedule)

- **Routine vaccination**:
    - Routine offer at 65 and 70 years of age from 1 September 2023, moving in stages towards a routine offer at 60. Anyone previously eligible remains eligible until their 80th birthday; where the first dose was given before 80, the second dose is given before the 81st birthday.
    - A **catch-up programme** includes those turning 66 to 70 years between 1 September 2023 and 31 August 2028.
- **Severely immunosuppressed individuals:**
    - From 1 September 2025 all severely immunosuppressed individuals aged 18 years and over are eligible for Shingrix on the NHS, with no upper age limit. This PGD covers them from 18 to 49 under Arm 2 and from 50 under Arm 1.
    - Green Book chapter 28a, Box 1 (severe immunosuppression): acute or chronic leukaemia or clinically aggressive lymphoma less than 12 months from cure; chronic lymphoproliferative disorder or haematological malignancy under follow-up (indolent lymphoma, chronic lymphoid leukaemia, myeloma, Waldenstrom's macroglobulinaemia, other plasma cell dyscrasias); HIV with a current CD4 count below 200 cells per microlitre; primary or acquired cellular or combined immune deficiency (lymphocytes below 1,000 per microlitre or a functional lymphocyte disorder); stem cell transplant in the previous 24 months, or longer ago with ongoing immunosuppression or graft versus host disease; immunosuppressive chemotherapy or radiotherapy in the past 6 months; immunosuppressive therapy for a solid organ transplant in the past 6 months; targeted therapy for autoimmune disease in the past 3 months (JAK inhibitors, biologic immune modulators, TNF inhibitors, IL-6, IL-17, IL-12/23 or IL-23 inhibitors; B-cell therapies such as rituximab count for 6 months); chronic immune-mediated inflammatory disease treated with prednisolone 20mg or more a day for more than 10 days in the past month, or 10mg or more a day for more than 4 weeks in the past 3 months, or a non-biological immune-modulating drug above the Box 1 threshold (methotrexate over 20mg a week, azathioprine over 3mg/kg/day, mercaptopurine over 1.5mg/kg/day, mycophenolate over 1g/day) in the past 3 months, or a Box 1 combination (prednisolone 7.5mg or more a day with another immunosuppressant other than hydroxychloroquine or sulfasalazine; methotrexate with leflunomide); or a short course of prednisolone over 40mg a day for more than a week for any

reason in the past month. Short high-dose courses of up to 40mg prednisolone a day for acute asthma, COPD or COVID-19, replacement corticosteroids, and topical or inhaled corticosteroids do not count. Primary humoral immunodeficiencies without a T-cell defect are not an indication without immunologist advice. If in doubt, discuss with the specialist. A patient who has had 2 doses of Shingrix does not need the course repeated on becoming immunosuppressed; a Zostavax recipient who becomes severely immunosuppressed is offered 2 doses of Shingrix.

- **Unvaccinated individuals aged 70 to 79 who missed the earlier programme remain eligible for Shingrix on the NHS.**

Dosing Schedules

- **Shingrix®**:
  - **Two doses of 0.5 mL. Aged 50 and over: second dose 2 to 6 months after the first (SmPC). Severely immunosuppressed: second dose 8 weeks to 6 months after the first (Green Book; the SmPC permits 1 to 2 months). If the course is interrupted or delayed, give the second dose as soon as possible and do not repeat the first dose (Green Book, previous incomplete vaccination).**
  - **NHS entitlement: a patient who is eligible for Shingrix on the NHS (aged 65 to 79, or severely immunosuppressed and aged 18 or over) is told so before a private supply proceeds, and the record shows this. NHS eligibility is not a condition of supply under this PGD, which follows the licensed indication.**

Contraindications

- **Shingrix**:
  - Hypersensitivity to any component.

Precautions

- Delay vaccination in cases of acute illness with fever.
- Not for treatment of acute shingles.
- Check vaccine history prior to administration.
- Pregnancy and breastfeeding: the Green Book states there is no known risk from giving a non-live vaccine in pregnancy or while breastfeeding, and that, if indicated, Shingrix can be considered in pregnancy after full discussion of the risks and benefits with the recipient. Inadvertent administration in pregnancy is reported to UKHSA on the vaccine in pregnancy (ViP) form.

Common Side Effects

- **Shingrix**: Localised pain, fatigue, headache, myalgia, fever.

Effectiveness

- **Shingrix**: Over **90% effective** at preventing shingles and PHN.

- Record date, site, batch number, expiry date and brand of vaccine, the dose number and the arm; for Arm 2 the Box 1 category and the condition or therapy relied on.
- Report any suspected adverse reactions to the **Yellow Card Scheme**.

Patient Group Direction

for the supply/administration of

Shingrix

for vaccination against

Shingles

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	● Pharmacist registered and practicing with the GPhC ● Pharmacy technician registered and practicing with the GPhC
Training and assessment	• Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. ● All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines ● All users must have completed training in working under Patient Group Directions, for example the CPPE PGD e-learning and e-assessment or an equivalent, and have been assessed as competent to work under a PGD before first working under this one; previous experience of working under a PGD is not required ● All users should complete the Get Real Health training for this PGD before first working under it, or have completed equivalent training elsewhere, for example another provider's training for the same service or substantial recent experience of delivering it under a PGD, and must read this PGD in full before first working under it. The date the training was completed, and for equivalent training what it was, is to be recorded in the Get Real Health training record or in the employing organisation's own training record. The employing organisation remains responsible for being satisfied that each practitioner it authorises under this PGD is competent to do so. ● Must work in compliance with the SOPs of their own employer ● All users must have appropriate indemnity in place to cover this service

Further training and responsibilities	● All users must be up to date with CPD requirements and appraisal ● All users must be up to date with MHRA and safety alerts with regards to medical products ● All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Shingrix is indicated for the prevention of herpes zoster (shingles) and herpes zoster-related postherpetic neuralgia in adults aged 50 years and older, and in adults aged 18 years and older at increased risk of herpes zoster. This PGD has two arms. Arm 1: adults aged 50 years and over. Arm 2: adults aged 18 to 49 years who are severely immunosuppressed as defined in Green Book chapter 28a, Box 1.
Inclusion criteria	Both arms: - Has not completed a two-dose course of Shingrix. Dose 2 may be given under this PGD where dose 1 was given elsewhere; record the date of dose 1. Previous Zostavax is not an exclusion. - No history of shingles in the past 12 months. - Informed consent obtained. - Where the patient is eligible for Shingrix on the NHS (aged 65 to 79, or severely immunosuppressed and aged 18 or over), they have been told so before a private supply proceeds, and this is recorded. NHS eligibility is not otherwise a condition of supply. Arm 1 (aged 50 and over): - Individuals aged 50 years or older, within the licensed indication. Arm 2 (aged 18 to 49, severely immunosuppressed): - Individuals aged 18 to 49 years who are severely immunosuppressed as defined in Green Book chapter 28a, Box 1 (severe immunosuppression): acute or chronic leukaemia or clinically aggressive lymphoma less than 12 months from cure; chronic lymphoproliferative disorder or haematological malignancy under follow-up (indolent lymphoma, chronic lymphoid leukaemia, myeloma, Waldenstrom's macroglobulinaemia, other plasma cell dyscrasias); HIV with a current CD4 count below 200 cells per microlitre; primary or acquired cellular or combined immune deficiency (lymphocytes below 1,000 per microlitre or a functional lymphocyte disorder); stem cell transplant in the previous 24 months, or longer ago with ongoing immunosuppression or graft versus host disease; immunosuppressive chemotherapy or radiotherapy in the past 6 months; immunosuppressive therapy for a solid organ transplant in the past 6 months; targeted therapy for autoimmune disease in the past 3 months (JAK inhibitors, biologic immune modulators, TNF inhibitors, IL-6, IL-17, IL-12/23 or IL-23 inhibitors; B-cell therapies such as rituximab count for 6 months); chronic immune-mediated inflammatory disease treated with prednisolone 20mg or more a day for more than 10 days in the past month, or 10mg or more a day for more than 4 weeks in the past 3 months, or a non-biological immune-modulating drug above the Box 1 threshold (methotrexate over 20mg a week, azathioprine over 3mg/kg/day, mercaptopurine over 1.5mg/kg/day, mycophenolate over 1g/day) in the past 3 months, or a Box 1 combination (prednisolone 7.5mg or more a day with another

	immunosuppressant other than hydroxychloroquine or sulfasalazine; methotrexate with leflunomide); or a short course of prednisolone over 40mg a day for more than a week for any reason in the past month. - The condition or therapy relied on, and its dates, are documented in the record. Where there is any doubt whether the Box 1 definition is met, the treating specialist or GP has confirmed it before vaccination.
Exclusion criteria	Both arms: - Hypersensitivity to any component of the vaccine. - Pregnancy and breastfeeding are not exclusions: see the cautions row. Arm 2 only (refer to the GP or treating specialist): - Aged under 18 years. - Immunosuppression that does not meet the Green Book Box 1 definition, including short courses of prednisolone up to 40mg a day for acute asthma, COPD or COVID-19, replacement corticosteroids, and topical or inhaled corticosteroids. - Primary humoral immunodeficiency (for example X-linked agammaglobulinaemia) without a T-cell defect, unless an immunologist has advised vaccination. - Not yet immunosuppressed but about to start immunosuppressive therapy: the treating specialist or GP can start the course before treatment on the Green Book timing (ideally one month, at least 14 days, before therapy). - Doubt whether the Box 1 definition is met that the specialist or GP has not resolved.
Cautions	Counsel patient that systemic side effects are common and generally self-limiting. In Arm 2 (and in immunosuppressed patients aged 50 and over) injection site pain, fatigue, myalgia, headache, shivering and fever are reported more often. Observe every patient for 15 minutes after vaccination, seated, and record that the observation period was completed (see Vaccine safety requirements). Allow appropriate spacing from other vaccines (e.g., COVID-19 or influenza) based on clinical judgement. Arm 2: the immune response may be reduced; advise that protection may be limited. Give the second dose 8 weeks to 6 months after the first so that protection is not delayed. A second dose more than 6 months after the first is given as soon as possible; the course is not restarted and dose 1 is not repeated (Green Book chapter 28a). Pregnancy and breastfeeding, both arms: Shingrix is a non-live vaccine;

	the Green Book states it may be considered in pregnancy after discussion of the benefit and the lack of data; breastfeeding is not a contraindication. Record the discussion and the decision.
Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be accessed. Document any advice given and the decision reached. Inform or refer to the GP as appropriate

Details of the medicine

Name, form and strength of medicine	Shingrix
Legal category	POM
Storage	Store in a refrigerator (2°C–8°C). Do not freeze. Protect from light.
Route/method of administration	Intramuscular injection, preferably in the deltoid muscle.
Dose and frequency	0.5 mL dose given twice. Arm 1 (aged 50 and over): second dose 2 to 6 months after the first. Arm 2 (aged 18 to 49, severely immunosuppressed): second dose 8 weeks to 6 months after the first (Green Book chapter 28a; the Shingrix SmPC permits 1 to 2 months for immunosuppressed patients, but this PGD uses the Green Book interval). Both arms: where more than 6 months have elapsed since dose 1, give dose 2 as soon as possible and do not restart the course (Green Book). Record the interval.
Quantity to be administered and/or supplied	0.5 mL per dose; two-dose course required
Maximum or minimum treatment period	Two doses. Minimum interval after dose 1: 2 months (Arm 1) or 8 weeks (Arm 2); intended maximum 6 months. A second dose more than 6 months after the first is still given under this PGD, as soon as possible; the course is not restarted.
Adverse effects	Localised pain, redness, swelling at injection site, fatigue, headache, myalgia, shivering, fever, gastrointestinal symptoms. Refer to SmPC for full details.

Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	Record the following information: ● that valid informed consent was given ● name of individual, address, date of birth and GP

	● name of healthcare practitioner ● name and brand of medication ● date of supply dose, form and route quantity supplied ● advice given, including advice given if excluded or declines treatment ● details of any adverse drug reactions and actions taken supplied via PGD ● the arm under which the vaccine was given (Arm 1, aged 50 and over; Arm 2, aged 18 to 49 severely immunosuppressed), the dose number, the date of dose 1 and the interval between doses, and batch number and expiry date ● for Arm 2, the Green Book chapter 28a Box 1 category that applies and the condition or therapy relied on with its dates, and where there was doubt, the specialist or GP who confirmed it and when ● where the patient is eligible for Shingrix on the NHS, that they were told it is free of charge on the NHS before the private supply ● where the patient is pregnant or breastfeeding, the discussion held under the cautions row and the decision reached Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows: ● Adults aged 18 years and over - keep records for 8 years ● Children aged under 18 years -keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday)

Patient information

Written information to be given to patient or carer	Supply the patient information leaflet (PIL) provided with the medication.
Follow-up advice to be given to patient or carer	To seek medical advice if side effects are severe or last more than a few days, and to seek urgent help for any sign of an allergic reaction after leaving the pharmacy. Remind the patient of the date the second dose is due (2 to 6 months after dose 1 in Arm 1; 8 weeks to 6 months in Arm 2), and that a late second dose should still be given as soon as possible without restarting the course.

Key references

- NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions. Published March 2017 https://www.nice.org.uk/guidance/mg2
- NICE MPG2 Patient group directions: competency framework for health professionals using patient group directions. Updated March 2017 https://www.nice.org.uk/guidance/mpg2/resources
- UKHSA, Immunisation Against Infectious Disease (the Green Book), chapter 28a Shingles (herpes zoster); JCVI advice on the shingles programme; Shingrix Summary of Product Characteristics, current version
- British National Formulary (BNF) Key information on the selection, prescribing, dispensing and administration of medicines BNF (British National Formulary) | NICE
- Summary of Product Characteristics

Agreement to practise by the Registered Healthcare Professional

By signing this PGD you are confirming that you agree to its contents and that you will work within it. PGDs do not remove inherent professional obligations or accountability. It is the responsibility of each healthcare professional to practise only within the bounds of their own competence and professional code of conduct.

Name and address of pharmacy premises / clinic premises to which this PGD relates			
Name of healthcare professional	Job title	Registration number	Signature
Valid from date		Expiry date	
30/9/26		31/7/27	

PGD adoption and authorisation by the employer/clinical lead

The employer has ensured that the person using this PGD, has the knowledge and skills to safely provide the service which will encompass the supply or administration of a medication or vaccine.

This section must be signed by the superintendent or clinical lead responsible for this service. To be completed by the adopting pharmacy. These are the adopting organisation's own signatories and must not be Get Real Health staff.

Name of superintendent / clinical lead	Job title & name of organisation	Signature	Date
Name of professional group signatory	Job title & name of organisation	Signature	Date

Change history to version 001

Later versions are recorded in the Version and change record at the end of this document.

Version number	Change details	Date
001	Development & issue of new PGD Later versions: see the Version and change record at the end of this document.	25th June 2025

Vaccine safety requirements

The requirements on the following pages form part of this Patient Group Direction and must be met before any vaccine is administered under it.

Anaphylaxis: what must be in place before you vaccinate

ADRENALINE (EPINEPHRINE) 1 IN 1,000 INJECTION must be immediately available in the room where vaccination takes place, in date, together with a telephone.

A written anaphylaxis protocol consistent with current Resuscitation Council UK guidance must be available, and every person administering must be trained in the recognition and immediate management of anaphylaxis and hold current basic life support training.

Be able to distinguish a faint and a panic attack from anaphylaxis. Green Book chapter 8 sets out the clinical features of each. A faint after an injection is common; anaphylaxis is very rare.

Report any suspected anaphylaxis via the Yellow Card Scheme even where the diagnosis is uncertain.

Observation after vaccination

OBSERVE EVERY PATIENT FOR 15 MINUTES AFTER VACCINATION, SEATED.

Anxiety-related reactions including vasovagal syncope, hyperventilation and transient visual disturbance or paraesthesia can occur before or after any injection, and are commonest in adolescents. They are a response to the needle, not to the vaccine.

Have procedures in place to prevent injury from a faint.

Record that the observation period was completed.

Cold chain and excursions

Store at +2C to +8C in the original packaging to protect from light. Do not freeze, and discard any vaccine that has been frozen.

COLD CHAIN EXCURSION: any stock exposed to conditions outside +2C to +8C must be QUARANTINED and risk assessed in accordance with UKHSA Vaccine Incident Guidance before any further use.

Do not administer excursion stock until that assessment is complete and has been recorded.

Where the product SPC gives specific stability data for a temporary excursion, that data governs; otherwise quarantine and assess.

Disposal

Dispose of used syringes, needles, vials and any unused or reconstituted vaccine in a UN-approved puncture-resistant sharps container.

Follow local authority requirements and Health Technical Memorandum 07-01, Safe management of healthcare waste.

Never re-sheath a needle.

Consent in children and young people

Where the individual is UNDER 16, valid consent must come from a person with PARENTAL RESPONSIBILITY, or from the young person where you assess them as GILLICK COMPETENT.

Record which of the two applied. Where consent was given by a person with parental responsibility, record their name and relationship to the patient. Where the young person consented on the basis of Gillick competence, record the basis of that assessment.

A parent accompanying a child does not automatically hold parental responsibility. Ask.

Users must be competent in assessing consent in children and young people before working under this PGD.

Version and change record

Version: v010

Issued: 30 September 2026

Supersedes: Version 009, 14 September 2026

Valid from: 30 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Training before first use, applied to every Get Real Health PGD on 30 September 2026: the staff section now states that each practitioner should complete the Get Real Health training for this PGD, or equivalent training elsewhere, before first working under it, must read the PGD in full before first use, and that the date of completion is recorded. Training is recorded; it is not a condition of access, and the employing organisation remains responsible for being satisfied of competence. No clinical content changes.

Previous version records

Version: v009

Issued: 14 September 2026

Supersedes: Version 008, 11 September 2026

Valid from: 14 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Identity and training, applied to every Get Real Health PGD on 14 September 2026 after two adopting pharmacies wrote: the Get Real Health logo is in the page header; the organisation that owns and authorises this direction, Get Real Health Limited, is named under the strapline with its company number and registered office; the requirement that practitioners must previously have used a PGD is replaced by a requirement for PGD training and a competence assessment, so that a newly registered pharmacist or technician who has completed the training may work under it; and the earlier change history table is headed with the versions it covers.

Previous version records

Version: v008

Issued: 11 September 2026

Supersedes: Version 007, 11 September 2026

Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Decision, joint sign-off 11 September 2026: pregnancy and breastfeeding move from the exclusion criteria to the cautions in both arms: Shingrix is a non-live vaccine; the Green Book states it may be considered in pregnancy after discussion of the benefit and the lack of data; breastfeeding is not a contraindication; the discussion and the decision are recorded, with a matching records bullet; the guidance summary no longer lists pregnancy as a contraindication and instead summarises Green Book chapter 28a (12 August 2025) on pregnancy and breastfeeding, including reporting of inadvertent administration in pregnancy to UKHSA; nothing else in the exclusion criteria changes; the consultation tool no longer stops on pregnancy or breastfeeding and instead requires the discussion and decision to be recorded and prints them on the record.

Joint sign-off by the Medical Director and the Head Pharmacist, 11 September 2026: the drafted texts of version 007 were reviewed together and confirmed, with the change above.

Previous version records

Version: v007

Issued: 11 September 2026

Supersedes: Version 006, 11 September 2026

Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Decision 13: a second dose more than 6 months after the first is now given as soon as possible without restarting the course (Green Book chapter 28a, previous incomplete vaccination), stated in the guidance summary, cautions, dose and frequency, treatment period, records and follow-up rows; the tool no longer stops on a late second dose and instead records the interval.

Decision 14: the inclusion bullet 'eligible under national immunisation guidelines' is replaced by the licensed indication (50 and over) with a requirement that an NHS-eligible patient (aged 65 to 79, or severely immunosuppressed and aged 18 or over) is told Shingrix is free on the NHS before a private supply, with a matching records bullet and guidance summary note.

Decision 15: Arm 2 added for adults aged 18 to 49 who are severely immunosuppressed as defined in Green Book chapter 28a Box 1 (inclusion listing the Box 1 categories, exclusions for immunosuppression outside Box 1, primary humoral immunodeficiency without a T-cell defect, anticipated therapy and unresolved doubt, two 0.5 mL doses 8 weeks to 6 months apart per the Green Book and the Shingrix SmPC, cautions on reduced response, and records of the Box 1 category and the condition or therapy relied on), written into the indication, inclusion, exclusion, cautions, dose, treatment period, records, follow-up and guidance summary rows; drafted from Green Book chapter 28a (12 August 2025) and the Shingrix SmPC for Chris Pilkington to confirm.

Decisions of the authorising signatories, 11 September 2026: the questions raised by the reviews of 10 and 11 September were put to the Medical Director and answered; each answer is written into this document above and, where the answer set a rule, into the consultation tool. Text drafted from a source for the Head Pharmacist to confirm is marked as such in the change lines.

Previous version records

Version: v006

Issued: 11 September 2026

Supersedes: Version 005, 11 September 2026

Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Guidance summary no longer describes Zostavax, which was withdrawn from the UK programme in September 2023 and is not supplied under this PGD; the 70 to 79 catch-up now refers to Shingrix.

Cautions row now states the 15 minute seated observation that the vaccine safety requirements already impose.

Follow-up advice row replaced the treatment template wording (3 to 4 weeks) with vaccination advice and a reminder of the second dose date.

Change history table now points to the version and change record for versions after 001.

Wording corrections from the tool alignment and adversarial review of the consultation tools, 11 September 2026. Each correction resolves a contradiction inside the document or a plain error, taking the safer reading; no indication is widened, no dose raised and no exclusion removed. Questions that need a clinical decision are recorded separately and are not changed here.

Previous version records

Version: v005

Issued: 11 September 2026

Supersedes: Version 004, 10 September 2026

Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

The guidance summary and references name Green Book chapter 28a, JCVI and the Shingrix SmPC as the governing sources.

Structural clean-up applied to every live Get Real Health PGD on 11 September 2026, following the adversarial review of 10 September: the adopting pharmacy's authorisation block is blank, for the pharmacy's own signatories; one signed authorisation page for this version replaces the earlier signature blocks; every validity cell states this version's dates (valid from 11 September 2026, expiry 31 July 2027); narration about earlier versions, the consultation tool and individual customers is removed from the body of the document; the change records of earlier reissues are carried below as previous version records. No clinical criterion changes unless listed.

Previous version records

Version: v002

Issued: 9 September 2026

Supersedes: the previous signed version of this document.

Change: the vaccine safety requirements above were added following an estate-wide sweep of every vaccination PGD, which found that the signed documents were missing, between them, any requirement for adrenaline to be available, any anaphylaxis provision, a stated observation period, a cold chain excursion procedure, a sharps disposal provision, a batch number requirement, and any wording on consent in children and young people. The specific items added to THIS document are: Anaphylaxis: what must be in place before you vaccinate, Observation after vaccination, Cold chain and excursions, Disposal, Consent in children and young people.

Authorised by Nitin Shori, Medical Director (GMC 6047293) and Chris Pilkington, Head Pharmacist (GPhC 2046322), on 9 September 2026. Signatures applied digitally on their joint instruction, which is the established process for Get Real Health PGDs.

Version: v003

Issued: 10 September 2026

Supersedes: Version 002, 9 September 2026

Valid from: 10 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Validity: this version is valid from 10 September 2026 and expires on 31 July 2027. The signed master previously stated no period of validity, or an expiry of 31 October 2026; those are superseded. No PGD-level period of validity was stated.

Inclusion: 'no previous shingles vaccination' excluded every patient attending for dose 2 of the two-dose course the PGD authorises, and any Zostavax recipient who now needs Shingrix. Reworded.

Version: v004

Issued: 10 September 2026

Supersedes: Version 003, 10 September 2026

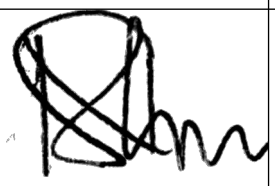
Valid from: 10 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Guidance summary: it stated that immunocompromised adults aged 18 and over are eligible for Shingrix, which the SmPC and Green Book support, while the PGD covers 50 and over only. The summary now says so. Whether to add an 18 to 49 immunosuppressed arm is recorded for decision.

Signed on behalf of Get Real Health

Version v010, 30 September 2026.

Name	***Qualification***	***Signature***	***Date***
Nitin Shori	Medical Director GMC: 6047293		30 September 2026
Chris Pilkington	Head Pharmacist GPhC: 2046322		30 September 2026

Both authorising signatories reviewed this version together on 30 September 2026 and gave their authorisation for it to be issued. Signatures were applied digitally on their joint instruction, which is the established process for Get Real Health PGDs. This version is not valid without both signatures.